

إِسْمُ الْمَرِيضِ رُبَاعِيًّا : نَبِيلُهُ عُمَرُ عَبْدِ اللَّهِ -

العنوان : الساحل

رقم البطاقة : 26802120102149 رقم التلفون : 01110454667

العمر(تاريخ الميلاد) : 12-02-1968 المهنة : حاصل على دبلوم

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

GRI-E

جورستان
روستا
لیدی
موسا
روسای

صحة الامانات المرفعة
اللجنة و مدير

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس

وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

قسم :

نَفَقَات :

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

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Name: نبيله عمر عبدالله - National ID: 26802120102149	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



تبيته
عمر عبدالله عيسى
ه ش الشعبين عبيد
الساحل - القاهرة

١١١١/٠١

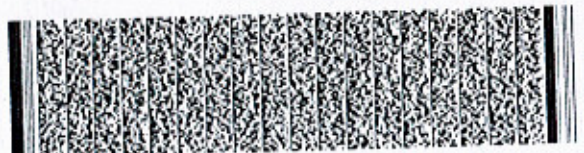
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حاصلة على دبلوم تجارة
أنثى
مسلمة
أرملة
البطاقة سارية حتى ٢٠٢٢/٠٩/٠٥



Patient Name	: الأستاذة / نبيلة عبد عبد الله	Request No.	: 13002389
Patient No.	: 26802120102149	Request Date	: 28/10/2024
Path No.	: 4137/24	Verification Date	: 13/11/2024
Sex	: Female	Financial Category	: أورام دار السلام - متفرده
Age	: 56 Y	Department	: Out Patient
Referring Doc	:	Registered by	: ..marwa mohamed youss

Pathology Report

Clinical Data: Left breast malignant looking solid mass lesion.

Nature of Specimen: Tru-cut biopsy, collected at 27/10/2024.

Gross Examination:

Three grayish white tissue cores up to 1 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angiolinvasion: No.
- * Microcalcifications: Un detected.
- * Additional pathological findings: Focal necrosis, marked desmoplasia
- * IFT (Positive control showed appropriate reactivity):
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
 - PR: Strong nuclear reaction in 80% of tumor cells (score 8/8).
 - Her2: No membrane reaction of tumor cells (score 0).
 - Ki67: 15% nuclear reaction of tumor cells.

Diagnosis:

Left breast malignant looking solid mass lesion, Tru-cut biopsy:

Verified by :

Dr.Toka Medhat

Printing Date : 18/11/2024 09:25:44

Page 1 of 3

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - قصر الندوة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mār ElQadema - Cairo, Egypt

Name: نبيله عمر عبدالله		
MR. : 4850		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
إيماء للطلب المقدم من	بشأن تقرير طبي للمريض	
بقسم:	رقم طبي:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
تاريخ وساعة الدخول :	تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه ER AND PR 8/8+HER2 -VE. UPFRONTLT BCS +SLND (PT2 PN0 M0). TODAY :ZOMETA +FEMARA +SUPPOURTIVE femara +supportive*3m sm ..pending next visit :18/10/2026 ER AND PR 8/8+HER2 -VE. UPFRONTLT BCS +SLND (PT2 PN0 M0). 8/2026 sm :free TODAY : FEMARA +SUPPOURTIVE *3m		
تحريراً في ,		
التاريخ :	الوقت :	20-09-2026
الطبيب المعالج :		نهى سعيد علي محمد
يعتمد ,		

Patient Name : الأستاذة / توبله عمر عبد الله	Request No. : 13002389
Patient No. : 26802120102149	Request Date : 28/10/2024
Path No. : 4137/24	Verification Date : 13/11/2024
Sex : Female	Financial Category : أورام دار السلام - مبرور
Age : 56 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

Invasive duct carcinoma, Grade II, with focal necrosis and marked desmoplasia.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 15% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Salma Yarek.

Dr. Samia Ali.

Verified by :

Dr. Toka Medhat

Printing Date : 13/11/2024 09:25:44

Page 2 of 3

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CAP
accredited

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr ElQadima - Cairo, Egypt

Patient Name : الأستاذة / فريدة عمر محمد الله

Request No. : 13000214

Patient No. : 26802120102149

Request Date : 31/12/2024

Path No. : 5271/24

Verification Date : 15/01/2025

Sex : Female

Financial Category : أورام دار السلام - مياكره

Age : 56 Y

Department : Out Patient

Referring Doc :

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Up-Front..

Nature Specimen: Left breast conservative surgery.

Gross Examination:

* Procedure: Left BCS

Fibrofatty tissue piece measured 14x9x4 cm with covering skin ellipse measured 15x4.5 cm.

* Tumor extent: A greyish white tumor bed site

* Size: 3.5x2x2 cm.

* Skin involvement: Unremarkable grossly

* Distance from margins: adjacent with no distance in between.

* there was a greyish white area measured 2x1x1 cm, away from medial margin by 0.5 cm

* SNLs: FFT piece measure 2x2x1 cm entangling 8 nodules up to 1.5x1 cm.

Microscopic Examination:

* Histological type: Invasive duct carcinoma with focal DCIS

* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II(3+2+1)

* Ductal carcinoma in situ: Focal

* Skin involvement: Free nipple & areola

* Resection margins: Free

* Microcalcifications: Not detected

* SNLs: No metastatic deposits 0/8

Diagnosis:

Left BCS, Up-front:

Invasive duct carcinoma with focal DCIS, Grade II

- Free Resection Margins

- Free nipple & areola

- SNLs: No metastatic deposits 0/8

Verified by :

Dr.Heba Hasan

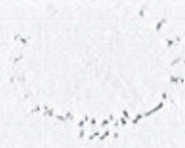
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 Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt



Patient Name: Nabila Omar Abd Allah Eisa	Date : 18/11/2024
Exam : C.T ABDOMEN & PELVIS	Age : 57 Year(s)
Medical ID: 4031/2024	Pat. ID: 93472

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Left breast LOQ mass lesion measures about 2x1.5cm.
- Multiple small level I and II axillary lymph nodes ,the largest is seen at level I measures about 1.6cm ,likely non specific.
- Left small supradicular lymph node measures about 1.5cm.
- Multiple small sub cm bilateral pulmonary nodules ,the largest is seen at right lower lung lobe posterior basal segment measuring about 5mm.
- No areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- The liver is enlarged (20cm) in size with homogenous CT texture. No definite focal lesions (on non contrast base) or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Gall bladder is not seen (surgical removed).
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Left adrenal anterior limb solid lesion measures about 1.3x2.1cm with HU -5.1 ,likely lipid rich adenoma.
- Normal appearance of the pancreas, aorta, I.V.C and right adrenal.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD is seen in place.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:** no definite worrisome lytic or sclerotic focal lesions.

Opinion:

- Multiple bilateral subcentimetric pulmonary nodules for follow up.
- No evidence hepatic metastatic deposits.

Dr.Aya Youssif

Dr. Mohamed Mohsen, M.D.

Patient's Name: Nabila Omar Abdallah
Date: 2026-08-22
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

٤٨٥٠

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Post left CBS.

Findings:

Right breast.

- Predominant fatty breast . (ACR A)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Predominant fatty breast . (ACR A)
- Thickened skin and coarsen trabeculae.
- architectural distortion with no speculations, masses, asymmetry , or suspicious microcalcifications.
- Ultrasound demonstrated parenchymal disruption underneath the operative scar with no solid , cystic lesions or fluid collections.
- Thickened skin and coarsen trabeculae.
- Normal nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- *Right breast (BIRADS 1): Normal study.*
- *Left breast (BIRADS 2): Post-therapeutic sequelae.*

Prof.MONA ZAKY

Dr Amal Hegazy

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